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TCGA-EM-A3AN-01A-PR

Redacted



Surgical Pathology Consultation Report

* Amended *

Patient Name:

MRN:

DOB:

Gender:

HCN:

Ordering MD:

Copy To:

Service:

Visit #:

Location:

Facility:

Accession #:

Collected:

Received:

Reported:

Specimen(s) Received

1. Neck: Right neck contents single stitch marks superior, double stitch marks anterior
2. Lymph node: Left jugulodigastric node
3. Neck: Left neck contents stitch marks apex
4. Parathyroid: Right upper parathyroid/QS
5. Lymph node: Left paratracheal node
6. Thyroid: Total thyroid, stitch marks upper pole, left lobe, with left paratracheal node

Diagnosis

1. Metastatic papillary thyroid carcinoma with extranodal extension: Lymph nodes, 12 of 37 (Right neck) dissection specimen
 2. No pathological diagnosis: Lymph node, 1 (left jugulodigastric) excision
 3. Metastatic papillary thyroid carcinoma: Lymph nodes, 7 of 31 (left neck) dissection specimen
 4. No pathological diagnosis: Parathyroid (right upper) biopsy
 5. Metastatic papillary thyroid carcinoma with extranodal extension: Lymph node, 1 of 2 (left paratracheal) excision
 6. Widely invasive papillary carcinoma, 1.2 cm, right, mixed classical and follicular variant, with psammomatous intrathyroidal dissemination involving isthmus and left; and focal involvement of right resection margin; mild nonspecific chronic thyroiditis; mild follicular nodular disease: Thyroid
- Metastatic papillary thyroid carcinoma with extranodal extension: Lymph nodes, 7 of 9, isthmus and central neck
 No pathological diagnosis: parathyroid, central compartment
 No pathological diagnosis: Thymus
 - total thyroidectomy and central compartment dissection specimen. See Comment.

Comment

The papillary carcinoma is widely invasive with psammomatous dissemination in the right and left lobes, filling lymphatic channels throughout the thyroid and in adjacent peri-thyroidal soft tissue. The largest single focus of tumor measures 1.2 cm and is in the right lobe. Although there are foci that are suspicious of angioinvasion, no definitive vascular invasion is identified. Because the tumor involves the painted margin of resection, the presence of extrathyroidal extension cannot be accurately assessed. Clinical correlation is recommended.

Synoptic Data

Procedure:

Total thyroidectomy with bilateral neck dissection

Other: Total thyroidectomy with bilateral neck dissection and central compartment dissection.

CCLF:
 path:

1CD-0-3
 carcinoma, papillary, thyroid, NOS 8240/3
 carcinoma, papillary, follicular variant 8340/3
 Site: thyroid, NOS C73.9 lw
 10/21/11

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
THPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 10/22/11	

Received:	Fresh
Specimen Integrity:	Intact
Specimen Size:	Right lobe:
	4.0 cm
	2.7 cm
	1.3 cm
	Left lobe:
	4.0 cm
	2.7 cm
	1.1 cm
	Isthmus +/- pyramidal lobe:
	2.0 cm
	0.8 cm
	0.5 cm
	Central compartment:
	4.5 cm
	2.5 cm
	1.5 cm
	Right neck dissection:
	14.8 cm
	4.3 cm
	1.5 cm
	Left neck dissection:
	11.0 cm
	3.0 cm
	1.8 cm
Specimen Weight:	22.3 g
Tumor Focality:	Unifocal
----- DOMINANT TUMOR -----	
Tumor Laterality:	Right lobe
Tumor Size:	Greatest dimension: 1.2cm
	Additional dimension: 1.0 cm
	Additional dimension: 1.0 cm
Histologic Type:	Papillary carcinoma, classical (usual)
	Papillary carcinoma, follicular variant
	Classical (papillary) architecture
	Follicular architecture
	Classical cytomorphology
Histologic Grade:	Not applicable
Margins:	Margin(s) involved by carcinoma
Tumor Capsule:	Totally encapsulated
Tumor Capsular Invasion:	Present, extent widely invasive
Lymph-Vascular Invasion:	Indeterminate
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Cannot be assessed
TNM Descriptors:	Not applicable
Primary Tumor (pT):	pT1b: Tumor more than 1 cm but not more than 2 cm in greatest dimension, limited to the thyroid
Regional Lymph Nodes (pN):	pN1b: Metastases to unilateral, bilateral or contralateral cervical or superior mediastinal lymph nodes
	Number of regional lymph nodes examined: 80
	Number of regional lymph nodes involved: 27
	Lymph Node, Extranodal Extension Present
Distant Metastasis (pM):	Not applicable
Additional Pathologic Findings:	Adenomatoid nodule(s) or Nodular follicular disease
	Thyroiditis, focal (nonspecific)
	Parathyroid gland(s), within normal limits
	Other: Thymus with no pathological diagnosis

per TSS, follicular variant 299% -
usual NPC

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Electronically verified by:

Gross Description

1. The specimen labeled with the patient's name and as "right neck contents, single stitch marks superior, double stitch marks anterior" consists of an oriented right neck dissection with overall dimensions of 14.8 cm SI x 1.5 cm ML x 4.3 cm AP. The specimen is painted with silver nitrate. The internal jugular vein, sternocleidomastoid and submandibular gland are not identified. Multiple lymph nodes ranging from 0.2 to 3.8 cm are identified; 4 of these are partially calcified. Tissue is stored frozen. Representative sections are submitted from superior to inferior:

- 1A multiple lymph nodes
- 1B one lymph node bisected
- 1C-D multiple lymph nodes
- 1E one lymph node bisected
- 1F representative section from largest partly calcified lymph node submitted for decalcification
- 1G 1 lymph node bisected
- 1H 1 lymph node serially sectioned
- 1I multiple lymph nodes
- 1J query one lymph node bisected
- 1K lymph node bisected
- 1L-M one lymph node bisected
- 1N multiple lymph nodes
- 1O one lymph node bisected
- 1P-R one partly calcified lymph node trisected and submitted for decalcification
- 1S multiple lymph nodes
- 1T-U one lymph node serially sectioned

2. The specimen labeled with the patient's name and as "left jugulodigastric node" consists of a piece of tan-brown tissue that measures 1.1 x 0.4 x 0.2 cm received in 10% buffered formalin.

2A submitted in

3. The specimen labeled with the patient's name and as "left neck contents, stitch marks apex" consists of an oriented left neck dissection with overall dimensions of 11.0 cm apex to base by 3.0 cm x 1.8 cm. There is a suture indicating the apex. The specimen is painted with silver nitrate. The internal jugular vein, sternocleidomastoid and submandibular gland are not identified. Multiple lymph nodes ranging from 0.3 to 3.0 cm are identified in multiple levels. Representative sections are submitted from apex to base.

- 3A-D multiple lymph nodes
- 3E one lymph node bisected
- 3F multiple lymph nodes
- 3G 1 lymph node bisected
- 3H multiple lymph nodes
- 3I-K one lymph node serially sectioned (K submitted for decalcification)

4. The specimen labeled with the patient's name and as "right upper parathyroid QS" consists of a piece of tissue that measures 1.1 cm in maximum dimension and weighs 0.004 g. It is frozen for intraoperative consultation.

4A frozen tissue resubmitted

5. The specimen labeled with the patient's name and as "left paratracheal node" consists of one piece of tan-brown tissue that measures 1.5 x 1.7 x 0.4 cm received in 10% buffered formalin. The specimen is painted with silver nitrate. Tissue is stored frozen.

5A submitted in

6. The specimen labeled with the patient's name and as "total thyroid, stitch marks upper pole, left lobe with left paratracheal node" consists of a thyroid gland that is oriented with a suture and weighs 22.3 g. Attached to the thyroid is a piece of soft tissue

WHICH OF THE FOLLOWING IS NOT A

Right upper parathyroid: Parathyroid tissue present called at _____

, MD